Campbell et al[1] are not merely limited to those of a single institution.

As part of the MMS quality-assurance protocol at Brown University, recurrent tumors are routinely logged and examined. We recently reviewed a total of 16 cases from 2005 through 2012 that were designated recurrent after previous MMS. These cases were selected based on availability of frozen-section slides and clinical photographs taken during the original MMS procedure and during treatment of the suspected recurrence. Critical examination of this information yielded several findings. In 1 case, a tumor labeled as recurrent was found to represent a different tumor type and location from the initial tumor treated. Thus, this lesion was reclassified as a separate primary. Further evaluation of clinical photographs revealed that 4 other tumors designated as recurrent may actually represent distinct primary lesions adjacent to and not overlying the initial surgery site. Histologically, 2 of these 4 lesions were infiltrative basal cell carcinomas. Compared with nodular basal cell carcinomas, infiltrative tumors typically present with more subclinical extension and skipped areas extending in a jagged and/or deeply infiltrative pattern.[4] It is possible that even though clinical photographs suggested that the tumor was a second primary located adjacent to the initial site of surgery, a skipped area in the tissue resulted in clear margins on the initial surgery.

Of the tumors considered definitive recurrences corroborated by clinical photographs and histology (n = 11), 2 lesions (18%) were found to have residual tumor at the margin on the initial surgery. This percentage is significantly higher than the previously cited 2.8% in the general MMS population of cleared tumors,[5] but is approaches that of the population of Campbell et al[1] of recurrent tumors. Three lesions (27%) in our cohort had either missing epidermis or dermal/subcutaneous tissue drop-out on the slides, which could lead to false interpretation of margin clearance. Our cohort also consisted predominately of basal cell carcinomas (n = 9).

In the context of these findings, it may be difficult to draw definitive conclusions based on our small sample size. However, we sought to draw increased awareness to factors contributing to tumor recurrences after MMS. Furthermore, quality-assurance protocols reviewing recurrent tumors may be helpful to the Mohs surgeon in determining reasons for recurrence.

Kachiu C. Lee, MD, MPH, H. William Higgins II, MD, MBE, and Raymond G. Dufresne, Jr, MD

Department of Dermatology, Brown University, Providence, Rhode Island

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Correspondence to: Kachiu C. Lee, MD, MPH, Department of Dermatology, Brown University, 593 Eddy St, APC #10, Providence, RI 02903

E-mail: kachiu@gmail.com

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Surgeon error and slide quality during Mohs micrographic surgery: Is there a relationship with tumor recurrence?

To the Editor: We would like to thank Dr Lee and colleagues for their comments and additional data regarding our study on slide quality and interpretation errors during Mohs micrographic surgery.[1] Their findings of an 18% incidence of unexcised marginal tumor in their recurrent Mohs tumors is in the realm of our incidence of 26% and that of Hruza[2] (20%). In addition, the 27% incidence of missing epidermal and/or subcutaneous drop-out of Lee et al also approximated our 21% finding. These data further emphasize the consequences of persistent tumor, and the possibility that the already excellent cure rate of Mohs micrographic surgery can be improved with more meticulous slide review. In addition, the high incidence of dermal tissue drop-out further validates this often-overlooked slide quality metric. Many focus exclusively on obtaining a complete epidermal margin, but missing dermal tissue also appears very important. We agree with Lee et al that a continuing quality improvement program might be a useful means to improve or maintain quality slide preparation and interpretation.

Tracy Campbell, MD,[a] April W. Armstrong, MD, MPH,[b] Clayton W. Schupp, PhD,[b] Keira Barr, MD,[b] and Daniel B. Eisen, MD[b]

Chicago Cosmetic Dermatology and Surgery, Illinois,[a] and Department of Dermatology, University of California Davis Medical Center, Sacramento[b]

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Correspondence to: Daniel B. Eisen, MD, Department of Dermatology, University of California Davis Medical Center, 3301 C St, #1400, Sacramento, CA 95816

E-mail: dbeisen@ucdavis.edu

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Generational gap regarding patients' driving habits

To the Editor: The article, "Sunscreen use while driving,"[1] serves as a valuable reminder of the importance of proper patient education regarding sunscreen use. The authors found that only 26% of the patients surveyed used sunscreen at least half the time while in the car. They drove the importance home by noting that 62% of the nonmelanoma skin cancers in their patients developed on the left side, compared with 28% on the right side ($P < .03$). This is a significant difference to note, especially considering that clear-glass and dark-tinted windows in cars only transmit 62% and 11.4% of ultraviolet (UV) A, respectively.

However, it is important to discuss a likely generational gap that exists between the patients they surveyed and current young drivers. The average patient age in the survey was 67.5 years, which means the majority of patients started their main driving years in the early 1960s. This is important since only in 1968, with the production of the American Motors Corporation's Ambassador, was air conditioning made standard in automobiles.[2] In 1969, only 54% of domestic automobiles were equipped with air conditioning.[3] In addition, 42.4% of adults were smokers in 1965, compared to 20.6% in 2008.[4] Considering both of these factors, it is likely that at least some of the patients surveyed drove with the windows down far more often than current drivers do, resulting in more exposure to UV radiation, particularly UVB, which is blocked by window glass. We thought this generational gap also deserved comment, as it may further have contributed to the effects of driving on UV light exposure and the left-sided dominance of actinic lesions in this US driving population.

Benjamin C. Garden, BA, and Patricia L. Myskowski, MD

Dermatology Service, Department of Medicine, Memorial Sloan-Kettering Cancer Center, New York, NY

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Correspondence to: Patricia L. Myskowski, MD, Memorial Sloan-Kettering Cancer Center, 160 E 53 St, New York, NY 10022

E-mail: myskowsp@MSKCC.org

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High survival rate of harlequin ichthyosis in Japan

To the Editor: Harlequin ichthyosis (HI) (OMIM 242500) is one of the most severe genetic skin disorders, and its clinical features at birth include severe ectropion, eclabium, flattening of the ears, and large, thick platelike scales over the entire body. Neonatal death is common.

We read with great interest the recent commentary by Milstone and Choate[1] on the importance of intensive neonatal care in improving HI outcomes. In their commentary, the authors conducted a critical examination of the intrinsic and extrinsic factors contributing to the prognosis of HI and they asserted that there are many effective measures for improving the outcomes of babies with HI.[1] For example, respiratory failure frequently contributes to neonatal death in HI[2]; thus, they noted that perhaps intubation should have been done in more cases.[1] A multicenter retrospective questionnaire-based survey in England, Sweden, the United States, Iran,